Lecture 20: The Good Life: Happiness

Introduction to Psychology · Yale University, Spring 2007 · Professor Paul Bloom

Study guide based on the lecture transcript. Source: https://oyc.yale.edu/psychology/psyc-110/lecture-20 (CC BY-NC-SA 3.0).

Before you start

In the last lecture of the course, Professor Bloom first finishes clinical psychology by asking whether therapy works. He then turns to positive psychology and research on happiness: how happy people say they are, why we get used to good and bad events, and how we remember pleasure and pain. He closes the course with two themes, humility and optimism.

Learning goals

After this lecture you should be able to:

- Explain how "regression to the mean" makes therapy hard to test, and how a control group helps.
- Describe the two "nonspecific factors" that help most therapies work.
- Explain why happiness ratings must be used with caution, using the dime and weather studies.
- Describe adaptation, the "hedonic treadmill" and affective forecasting.
- Explain what "happiness is relative" means and why "endings matter" when we remember pain.

Key ideas

1. How and Why Therapy Works

- Most people say they got better after therapy, but this could be **regression to the mean**. Moods go up and down around an average, and people go to therapy when they feel unusually bad, so they will probably feel better later even if therapy does nothing.
- Bloom's example: do naked jumping jacks on the worst day of your life. The next day will probably be better, but not because of the jumping jacks.
- The solution is a **control group** (people who get no treatment, for comparison). In one study, some depressed people were randomly put on a waiting list. Those who received cognitive training had lower depression scores than those with no therapy.
- Conclusions: therapy by and large works. Different therapies suit different problems: for unipolar depression, cognitive behavioral therapy and possibly antidepressant drugs (SSRIs); for bipolar depression, cognitive behavioral therapy is useless and medication is the best bet. And some therapists are better than others.
- Therapy also works because of **nonspecific factors**, properties almost all therapies share: **support** (someone really cares about you) and **hope**. Hope may work like a **placebo effect**: the belief that things will get better can make itself come true (a self-fulfilling prophecy).

2. Question and Answer on Therapy

Regression to the mean assumes that each person has a real average, such as a usual mood or "set point", and Bloom says this is plausible. Being told you are in a waiting-list control group could itself cause anxiety, so other studies use control groups that do not know they were not chosen.

3. Happiness and Positive Psychology

- Most of clinical psychology focuses on human misery. **Positive psychology** asks how to study people who are psychologically okay and make them better. Bloom thinks some of this work is bad, but a lot of it is interesting and promising.
- Asked "How happy are you, from one to ten?", most people answer seven or eight. There is a "Lake Wobegon effect": most people think they are happier than most people. Women are slightly happier than men on average, although they are more vulnerable to depression.
- In a study of forty-two countries, no country said it was unhappy. The Swiss were the happiest, the Bulgarians the saddest, and Americans scored 7.71.
- These numbers need caution. People who found a dime on a photocopy machine reported greater satisfaction with their whole life. People also report more happiness on sunny days, but this effect goes away if they are first asked about the weather.

4. Getting Used to Happiness

- From an evolutionary view, happiness is a **goal state**: a signal that our needs are satisfied. Steven Pinker says we are happier when we are "healthy, well-fed, comfortable, safe, prosperous, knowledgeable, respected, non-celibate, in love." But Americans today are healthier and better fed than people in the 1950s, and they are not happier.
- **Happiness doesn't change as much as you think.** It seems to have a strong **heritable** (genetic) basis, a "set point". About a year after being paralyzed from the neck down, people's happiness comes back to about where it was before; a year after winning the lottery, people are not as happy. Dan Gilbert's work shows we are bad at **affective forecasting** (predicting how happy or sad future events will make us), for example about getting tenure (a university job for life) or an election result. Reasons: many events do not matter in day-to-day life, and we **adapt** (get used to things).
- There are exceptions: we do not get used to noise, and cosmetic surgery makes people happier, and they stay happier. Adaptation is called the **hedonic treadmill**: you keep running but stay where you are. A king in the Bible book of Ecclesiastes had everything, yet said, "I hated life." One response is endless novelty; another is to step off the treadmill and seek friends, family and long-term projects.
- **Happiness is relative.** If you are desperately poor, you will not be happy, but beyond that what matters is your relative position. H.L. Mencken wrote, "A wealthy man is one who earns a hundred dollars more than his wife's sister's husband." People prefer $70,000 when others earn $65,000 to $75,000 when others earn $80,000.
- **Endings matter.** Daniel Kahneman found that memories of pleasure and pain are skewed to peaks and endings. An hour of painful dental work followed by a few minutes of mild pain is

remembered as less bad than the hour alone. He studied this with hands in freezing cold water and with a painful medical procedure (colonoscopy).

5. Closing Remarks

- Bloom closes the course with two themes. The first is **humility**: we know the brain is the source of mental life, but not how a physical object gives rise to conscious experience. About half of the differences in personality are due to genes, but we cannot explain the other half. We also do not know why some people resist bad social influences and do good things.
- The second theme is **optimism**: these questions will eventually be answered by building scientific theories (evolutionary, neurological, developmental, computational) and testing them with experiments and observation.
- Some people fear that science takes away what is special about people. Bloom disagrees: the more we study the mind scientifically, the more we appreciate its complexity, uniqueness and beauty.

Key vocabulary

Term	Meaning in simple English	Example from the lecture
regression to the mean	the tendency of unusual highs or lows to move back towards the average	Feeling better the day after the worst day of your life
control group	people in a study who do not get the treatment, for comparison	Depressed people put on a waiting list
nonspecific factors	helpful properties that almost all therapies share	Support and hope
placebo effect	improvement caused by believing a treatment will help	A therapist who dances while you watch
positive psychology	the study of how people can be at their best	*The Handbook of Positive Psychology*
set point	a person's natural, fairly fixed level of happiness	Identical twins are very similar in happiness
heritable	partly passed on through genes	Happiness has a strong heritable basis
affective forecasting	predicting how happy or sad future events will make you	Overestimating how bad an election loss will feel
adapt	to get used to a new situation	Getting used to a Nobel prize
hedonic treadmill	always adapting, so happiness does not increase	The king in Ecclesiastes
relative	compared with other people	$70,000 when others earn $65,000

Check your understanding

1. Why does "I felt better after therapy" not prove that therapy works?

2. How does a waiting-list control group help to test therapy?
3. What are the two nonspecific factors?
4. What do the dime study and the weather study show about happiness ratings?
5. What happens to people's happiness about a year after being paralyzed or winning the lottery?
6. What is affective forecasting, and what are two reasons we are bad at it?
7. In the lecture's salary example, which choice do people prefer, and what does this show?
8. Party A is great for 90% of the time but ends badly. Party B is bad for 90% but ends very well. Following Kahneman, why does B have a "huge pull" in memory?

Answer guide

1. People go to therapy when they feel unusually bad, and moods tend to return to average (regression to the mean). So they would probably improve anyway.
2. People at the same starting point are randomly given therapy or put on a waiting list, so the two groups can be compared.
3. Support (someone cares about you) and hope (the belief that you will get better).
4. Small, unrelated things (finding a dime, sunny weather) change how people rate their whole life, so we should be cautious about these ratings.
5. After paralysis, happiness comes back to about where it was before. A year after a lottery win, people are not as happy.
6. Predicting how happy or sad future events will make us. Many events do not matter in day-to-day life, and we adapt.
7. $70,000 when others earn $65,000. Happiness depends on our position relative to other people.
8. We do not just add up pleasure and pain. Memory is skewed to peaks and endings, and endings matter a lot.

English practice

Useful phrases from the lecture

- "It turns out that..." - introduces a surprising finding.
- "That doesn't mean... Rather, it just means..." - corrects a wrong conclusion.
- "Now, I don't want to overstate this." - warns that a claim has limits.
- "The idea is..." - explains the main point of a theory.
- "In the end, endings matter." - states a final conclusion.

Speaking task (with your AI tutor)

- Explain why big events do not change our happiness as much as we expect, in 4-6 sentences. Use: *affective forecasting*, *adapt*, *hedonic treadmill*.
- Explain why it is difficult to test whether therapy works, in 4-6 sentences. Use: *regression to the mean*, *control group*, *placebo effect*.

Writing task

- Write one paragraph (80-120 words): Explain Kahneman's finding that "endings matter". Use the dental example.

Study with your tutor

Things to say to your tutor in Live conversation:

- "Let's review Lecture 20. Ask me questions about regression to the mean and control groups, one at a time."

- "Quiz me on the key vocabulary of Lecture 20."
- "I will explain the hedonic treadmill in my own words. Check my understanding and my English."

Readings from the original course

- Csikszentmihalyi, Mihaly. "Flow: The Psychology of Optimal Experience." In The Norton Psychology Reader. Edited by Gary Marcus. New York: W. W. Norton & Company, 2006. pp. 210-222
- Sapolsky, Robert M. "Why Zebras Don't Get Ulcers: An Updated Guide to Stress, Stress-Related Diseases, and Coping." In The Norton Psychology Reader. Edited by Gary Marcus. New York: W. W. Norton & Company, 2006. pp. 223-232

My notes